



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : زيزى محمد بيومى عطيه
المحافظة : الشرقيه
الحى / المدينة : الزقازيق
العنوان :
رقم البطاقة : 26502041300101 رقم التليفون : 01065705007
العمر (تاريخ الميلاد) : 04-02-1965 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 21-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....)
المقر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي العلاج بعقار لاباتينب [14133] قسم العلاج : الاورام
التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: زيزى محمد يوزى عطيه
National ID: 26502041300101



تقرير طبي

المسيد

تحية طيبة وبعد,,

بشان تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بان نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
-- -- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Lapatinib therapy

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين



09/07/2026

MDT FORM

Name:	ZIZY Mohamed Byoumy Atiah	MRN:	4287
Age:	62 Y	ID:	26502041300101
Menopausal Sta	post	Phone:	01283570671
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:	htn	Reason Of MDT:	mbc

Clinical Data:

BX: IDC (PS2)
ER: 7/8 PR: 5/8 HER2: +3 KI67:

R

L

Sonomammography:

Mets Workup:

2/2023 denovo metastatic breast cancer to liver and lung
1st line chemo Paclitaxel + Trastuzumab IV
received 31 wks. of taxol+11 cycles of Herceptin

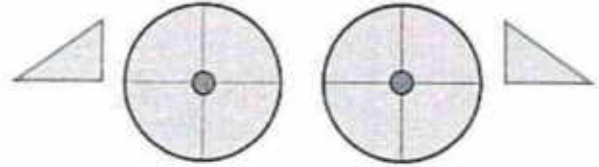
CT cap:SD

PATIENT SHOWED SD IN LUNGS, NEAR CR IN LIVER, PR IN LUNGS AND AXILLA
kept on maintenance Herceptin + AI+ zometa till 11/2024

11/2024 CT CAP: DP of HFLS

12/2025 mdt: 2nd line XELODA-TYKERB

7/2026 CT CAP: • Progressive course of the pulmonary metastases
• NO HFL !!



T N M

History:

Recommendations:

MDT Outcome & Plan

For trial of TDM1 through NGO....Meanwhile, shift to Gemzar/Carboplatin and maintain on Tykerb.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. شيماء لاشين	د. إلهام محمد				

2/5/2026
7/14/2026

Patient's Name:

Zizi

Mohammed Beyoumi Atieh



Date:

2026-08-13

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt



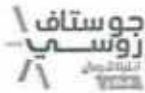
MULTISLICE CT EXAMINATIUN OF THE CHEST ,ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-

- Compare to previous study dated 15/4/2026, the current study showed
- An enhancing neoplastic mass lesion is seen in the deep upper outer quadrant of the left breast with central area of breaking down and /or cystic degeneration and measures about 24x21 mm in dimensions. Another spiculated lesion is also seen in the supra-areolar region of the same breast with foci of microcalcifications noted inside and measures about 36x26mm in dimensions .The overlying skin is thickened, and measures about 10 mm.
- Pathologically-enlarged left axillary lymph nodes are seen with loss of their hilar fat contents and the largest one measures about 14x11mm in dimensions.
- Normal right breast.
- Benign-looking right axillary lymph nodes are seen with preserved their hilar fat contents and the largest one measures about 7mm.
- Hyperinflated both lungs.
- Enhancing pretracheal and carinal/subcarinal lymph nodes are seen, and each globally measures measuring about 36x11mm and 37x17mm in dimensions.
- Progressive course in size and numbers of the multiple variable sized enhancing bilateral pulmonary nodules are seen with the largest one seen in the anterior segment of right lower pulmonary lobe and measures about 17 mm.
- Linear atelectatic changes are seen in the lingula and in both lower pulmonary lobes.
- No other patchy areas of consolidations or foci of calcifications noted.
- No sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusions seen.
- Average size of the heart with normal CT of its big vessels.
- Osteolytic bone lesion is still seen in the body of the sternum.

Patient's Name:

Zizi

Mohammed Beyoumi Atieh



Date:

2026-08-13

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Hospital:

Goustave Roussey International Egypt

- Average sized liver, measuring about 14.5 cm, showing a normal parenchymal attenuation and a homogenous enhancement pattern.
- No sizable abnormally-enhancing focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen, measuring about 9.5 cm. No focal lesions seen inside.
- Normal CT appearance of the pancreas, functioning both kidneys, both suprarenal glands, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones, solid masses, wall thickening or diverticulosis seen.
- A tiny left ovarian cyst is seen and measures about 18x18mm in dimensions.
- The rest of the pelvic structures are unremarkable.

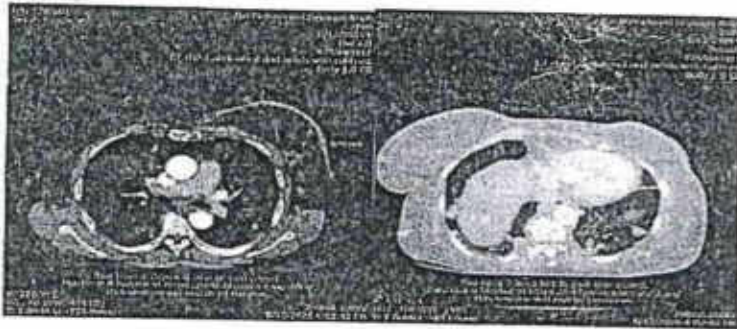
IMPRESSIONS:

- **Progressive course of the pulmonary metastases**
- **Stationary rest of study**

Prof/Hanan El-Ahwal.

Patient's Name: Zizi Mohammed Beyoumi Atieh
Date: 2026-08-13
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

د. كمال



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- No sizable abdominal or pelvic lymphadenopathy.

Patient Name : الأستاذ / زيزي محمد بيومي عطيه	Request No. : 13006458
Patient No. : 26502041200101	Request Date : 14/12/2022
Path No. : SR -973/22	Verification Date : 18/12/2022
Sex : Female	Financial Category : اورام دار السلام - نفقة الدولة
Age : 57 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

CLINICAL DATA : Right breast lesion

NATURE OF SPECIMEN

Revision of tru cut biopsy & IHC slides

GROSS EXAMINATION

One block + 5 slides .

MICROSCOPIC EXAMINATION

- * Histological type: Invasive duct carcinoma .
- * Grade (differentiation +nuclear pleomorphism+mitosis) : (3+2+2) Grade II
- * DCIS: No
- * Angioinvasion: Couldn't be detected in the received biopsy
- * IPT (Positive control showed appropriate reactivity) :
- * ER: Strong nuclear reaction in 60% of tumor cells (Allred score 7/8)
- * PR: Strong nuclear reaction in 12% of tumor cells (Allred score 5/8)
- * HER2: REPEATED
- * SISH: Amplified with cluster

DIAGNOSIS

Left breast mass ; slide revision of tru cut biopsy specimen :

Invasive duct carcinoma, Grade II .

- * ER: Positive (score 7/8).
- * PR: Positive (score 5/8).
- * HER2: REPEATED

Prof. Dr. Lubna Omer

Verified by :
saamah sayed

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Page 1 of 2

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Msr El Qadima - Cairo, Egypt



Patient Name	: الأستاذة / زيزي محمد بيومي عطيه	Request No.	: 13006458
Patient No.	: 26502041200101	Request Date	: 14/12/2022
Path No.	: SR -973/22	Verification Date	: 25/12/2022
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 57 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast lesion.
Nature of Specimen: Slide revision.

Gross Examination:

- One block + 5 slides.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for HER-2 clone (4B5).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma .
- * Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+2) Grade II
- * DCIS: No
- * Angioinvasion: Couldn't be detected in the received biopsy
- * IPT (Positive control showed appropriate reactivity) :
- * ER: Strong nuclear reaction in 60% of tumor cells (Allred score 7/8)
- * PR: Strong nuclear reaction in 12% of tumor cells (Allred score 5/8)
- * HER2: Strong complete membrane reaction in >10% of tumor cells (score 3+)
- * SISH: Amplified with cluster

Diagnosis:

Left breast mass, slide revision:
Invasive duct carcinoma, Grade II.

- * ER: Positive (score 7/8).
- * PR: Positive (score 5/8).
- * HER2: Positive (score 3+).

Prof. Dr. Lubna Omer

Verified by :
Dr.Amin Ahmed

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Page 1 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name	الأستاذة / زيزي محمد بيومي عطية	Request No.	13006458
Patient No.	26502041200101	Request Date	14/12/2022
Path No.	SR -973/22	Verification Date	25/12/2022
Sex	Female	Financial Category	اورام دار السلام - نفقة الدولة
Age	57 Y	Department	Out Patient
Referring Doc.		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Verified by :
Dr.Amin Ahmed

Printing Date : 22/08/2024 12:08:07

Page 2 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name: Zizi Mohamed Bayoumi Atteya	Date : 27/03/2025
Exam : C.T CHEST WITH CONTRAST	Age : 60 Year(s)
Medical ID: 4868/2022	Pat. ID: 31977

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

- Compared to the previous study dated 16-10-2024.

I Chest:

- Left breast shows stationary few parenchymal nodular densities and skin thickening & prominent left axillary lymph nodes are still noted.
- **Regressive course regarding** some previously noted few bilateral pulmonary nodules and the largest at lingula still measures 11 mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- average cardiac size No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and great vessels

II Abdomen & pelvis:

- Enlarged liver showing regressive **course** of size of previously noted segment IVb/V hypodense focal lesions the largest measures currently about 2.3x2.7 cm
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both suprarenals.
- Smooth filling of the urinary bladder.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Still noted uterine fundus small partially calcified lesion (fibroid).
- Stable sclerotic bone lesion in sternal body.
- Dorsal & lumbar vertebral spondylotic changes.

Opinion:

Status follow up showing:

- **Regressive course of previously noted some pulmonary nodules and hepatic focal lesions**
- **Rather stationary rest of the study.**

Dr. Ahmed Elgazar

Prof. Menna Tallah Hanafy, M.D.

Name: Mrs. ZIZI MOHAMED BAYOUMI

Date: 2026-09-02

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: Pathologically proven left breast cancer, post NACT assessment study with no available radiological data for comparison .

Findings:

Left breast.

- Scattered densities of glandular tissue (ACR B)
- An irregular shaped central dense mass with pleomorphic microcalcifications is seen ..
- Thickened skin.
- Retracted nipple .
- Ultrasound shows the pathologically proved malignant mass is seen infiltrating ,most of the inner half of the breast measuring (6 x 4.5 x 4 cm).
- Thickened skin reaching 8 mm with coarsened trabeculae and minimal interstitial as well as subdermal edema.
- Two small axillary lesions with calcifications ,the largest measures (15 x 14 mm) ...mostly pathological nodal with extranodal affection .. No pathological infra or supra-clavicular lymph nodes.

Right breast.

- Scattered densities of glandular tissue (ACR B)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated heterogeneous breast with mottled appearance with no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

Left breast (BIRADS 6): Locally advanced malignant mass .

Right breast (BIRADS 1): Normal

Best Regards,

DR, Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

<u>Category & recommendations</u>	<u>Suspected Cancer Risk</u>
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية



09/07/2026

MDT FORM

Name:	ZIZY Mohamed Broumy Atiah	MRN:	4287
Age:	62 Y	ID:	26502041300101
Menopausal Sta	post	Phone:	01283570671
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:	niln	Reason Of MDT:	mbc

Clinical Data:

BX:

IDC (PS2)

R

L

ER: 7/8

PR: 5/8

HER2: +3

KI67:

Sonomammography:

Mets Workup:

2/2023 denovo metastatic breast cancer to liver and lung
1st line chemo Paclitaxel + Trastuzumab IV
received 31 wks. of taxol+11 cycles of Herceptin

CT cap:SD

PATIENT SHOWED SD IN LUNGS, NEAR CR IN LIVER, PR IN LUNGS AND AXILLA
kept on maintenance Herceptin + AI+ zometa till 11/2024

11/2024 CT CAP: DP of HFLS

12/2025 mdt: 2nd line XELODA-TYKERB

7/2026 CT CAP: + Progressive course of the pulmonary metastases

- NO HFL !!

History:

Recommendations:

MDT Outcome & Plan

For trial of TDM1 through NGO... Meanwhile, shift to Genzar/Carboplatin and
maintain on Tykerb.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
أ.م.د. محمد زكي	أ.م.د. محمد زكي				

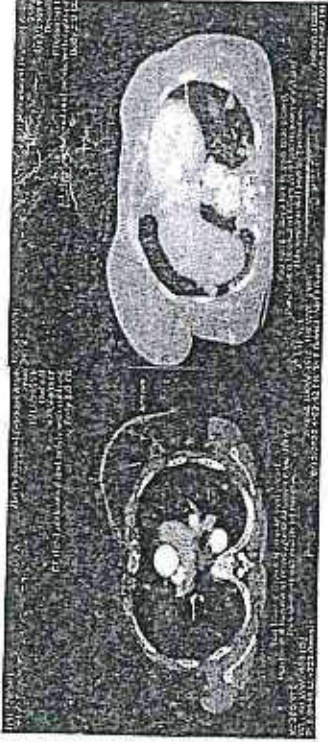
2/19/2026
Don't forget
2/19/2026



T N M

Patient's Name: Zizi Mohammed Beyoumi Atieh
Date: 2026-08-13
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

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MULTISLICE CT EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-

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- Normal right breast.
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- Linear atelectatic changes are seen in the lingula and in both lower pulmonary lobes.
- No other patchy areas of consolidations or foci of calcifications noted.
- No sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusions seen.
- Average size of the heart with normal CT of its big vessels.
- Osteolytic bone lesion is still seen in the body of the sternum.
- Average sized liver, measuring about 14.5 cm, showing a normal parenchymal attenuation and a homogenous enhancement pattern.
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- Patent PV.
- Average sized spleen, measuring about 9.5 cm. No focal lesions seen inside.
- Normal CT appearance of the pancreas, functioning both kidneys, both suprarenal glands, aorta and IVC.
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11/3/2026

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Zizi

Mohammed Beyoumi Atieh



Date:

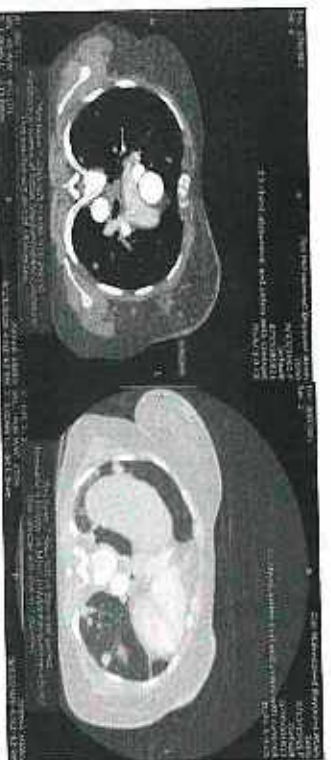
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Zizi

Mohammed Beyouni Ateih



Date:

2026-08-13

Referred by:

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- A tiny left ovarian cyst is seen and measures about 18x18mm in dimensions.
- The rest of the pelvic structures are unremarkable..

IMPRESSIONS:

- **Progressive course of the pulmonary metastases**
- **Stationary rest of study**

Prof./Hanan El-Ahwal.

Name: Mrs. ZIZI MOHAMED BAYOUMI

Date: 2026-09-02

Bilateral Digital soft tissue mammography with high resolution breast sonography

revealed:

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- Ultrasound shows the pathologically proved malignant mass is seen infiltrating ,most of the inner half of the breast measuring (6 x 4.5 x 4 cm),
- Thickened skin reaching 8 mm with coarsened trabeculae and minimal interstitial as well as subdermal edema.
- Two small axillary lesions with calcifications ,the largest measures (15 x 14 mm) ...mostly pathological nodal with extranodal affection .. No pathological infra or supra-clavicular lymph nodes.

Right breast:

- Scattered densities of glandular tissue (ACR B)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- Ultrasound demonstrated heterogeneous breast with mottled appearance with no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

Left breast (BIRADS 6): Locally advanced malignant mass .

Right breast (BIRADS 1): Normal

Best Regards,

DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

<u>Category & recommendations</u>	<u>Suspected Cancer Risk</u>
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	

PATIENT NAME Zeze MOHAMMED BAYYOMI ATEA	PATIENT ID #: 2022210311
GENDER: Female	ORDER #: 1989097,1989098
AGE: 57 y	DATE OF SERVICE: 16-Nov-2022 03:41 PM

Clinical indication - all studies

Report: - all studies

**FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY
ULTRASOUND****REPORT:**

- Scattered fibroglandular densities (ACR b).
- No speculated mass lesions or microcalcific clusters.
- Left breast UOQ pleomorphic calcifications are seen measuring about 3.1 x 4.2 cm corresponding on ultrasound to a para-areolar 2 o'clock ill defined lesion measuring about 3 x 2 cm with smaller nodules are seen surrounding it.
- Diffuse left breast skin thickening is seen measuring about 7 mm.
- Normal nipple areola complexes.
- Nonspecific right axillary lymphadenopathy.
- Left axillary pathologically enlarged left axillary lymph nodes are seen the largest measuring about 2.2 x 1.1 cm.

OPINION:

- *Left breast malignat looking masses as described above with pathologically enlarged left axillary lymph nodes (BIRADS 5).*

Recommendation - all studies
Referral to clinic

Reported By,

Dr. Ahmed Rozeka, MSc

16-Nov-2022 04:11 PM

Patient Name: Zizi Mohamed Bayoumi	Date : 18/09/2023
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 58 Year(s)
Medical ID: 4868/2022	Pat. ID: 31977

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS:

Compared to the previous study dated 22/06/2023.

I Chest: -

- Still noted left breast skin thickening with mild regression of the size of the enlarged left axillary pathological lymph nodes, the largest measuring about 1.1 x 1.7 cm.
- Stationary status of the bilateral few pulmonary nodules, the largest at lingula measuring 11 mm.
- A port a cath is still seen in place.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- Enlarged liver showing homogenous texture with no definite hepatic focal lesions or area of abnormal enhancement.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Still noted sclerotic bone lesion in sternal body.
- Dorsal and lumber vertebral spondylotic changes with osteophytic lipping.

Opinion:

- Status follow up showing overall stationary course since the last study.

Dr. Fifi Samy Mahmoud, MSc

Dr. Fifi Samy Mahmoud, M.D.



مبادرات رئيس الجمهورية
 لدعم صحة المرأة المصرية

السجل المصرية
 صفة
 مطر

رة شبي MDT FORM / 20 G
 15-11-2019

Age	MRN
Menopausal status	ID
NRS SCORE	Phone
BMI	Marital status
Comorbidity	Family history
	REASON OF MDT

58207-71

INDG II

7/8	5/8	HER 2: (+)	K157:
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A km: are of MSe → histology

received 31 wks of Herceptin + 17 cycles of Taxol

for consultation regarding maintenance and 2 Herceptin

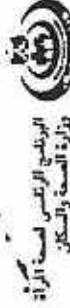
as C / CAP / SM

h/s

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric



Presidential Program for Women's Health
Ministry of Health and Population



MDT form

A-Patient ID:

1- Name : نوري محمد الهادي علي

Sex:

Female

3- Medical number:

83111230

Old Med :

2-ID number: 26502041300101

Patient Age : 58 Y



Family history

Phone

01060028001

BMI

REASON OF MDT

NEW CASE

marital status

HIS Score

GROUP

Date of MDT

01/02/2023

Co morbidity

marital status

Menopausal status

□

Clinical data

Breast Imaging :

Mets Workup :

CT CAP: Multiple HFLs and multiple lung mets

BX:

LT IDC GII 11/22//*/
AT BAHEYA

IHC:

ER:
7/8PR:
5/8Her 2 Neu :
+3

Ki67 :

T

N

3

M
M1 liver, lung

cT4d

Sonomammography

Recommendations:

Revise IHC

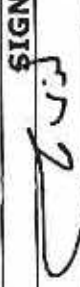
Echo. labs

Porta cath

Bone scan

MDT OUTCOME & PLAN

For 1st line chemo Paclitaxel + Trastuzumab IV if HER2 positive on revision

SIGNATURE		
MEDICAL ONCOLOGY		RADIOLOGY
SURGERY		PATHOLOGY
Psychiatric		Nutrition

about:blank



MDT form

A-Patient ID:

1- Name : زكري محمد بنومي علي

Sex:

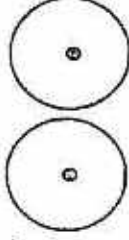
3- Medical number:
Female

83111230
Old Med :

2-ID number:

Patient Age : 59 Y

Old Med :
4867-2022



Family history	Family history	Phone	01200379422	BMI	54	REASON OF MDT	REASON OF MDT
marital status	Marital Status	NRS Score	01283570671	GROUP	GROUP	Date of MDT	Date of MDT
Co morbidity	Co Morbidity	Menopausal status	Menopausal Status	Initial STAGE	Initial STAGE	2024-11-25	2024-11-25

Breast Imaging :				Clinical data			
Mets Workup :				CT CAP: Multiple HFLs and multiple lung mets			
BX	ER	PR	Her 2 Neu	SISH	KI67		
IDC	7/8	5/8	3				
BY COMMENT:	PR COMMENT:	HER 2 Neu COMMENT:		SISH COMMENT:			
g11							
T	N	3					
CT4d							
Sonomammography							
M1 liver, lung							

PATIENT HISTORY

received 31 wks of taxol+11 cycles of herceptin
ct cap:SD

PATIENT SHOWED SD IN LUNGS, NEAR CR IN LIVER, PR IN LUNGS AND AXILLA
kept on maintenance herceptin + AI+ zometa till now

CT CAP: DP of HFLs

EF: 54%

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

For 2nd line XELODA-TYKERB-Zometa

MEDICAL ONCOLOGY	SIGNATURE		
	MEDICAL ONCOLOGY	RADIOLOGY	RADIOLOGY
SURGERY		PATHOLOGY	PATHOLOGY
Psychiatric		Nutrition	Nutrition

02/01/2023, 12:15

Cod 42 - Name_A MDT form Hermal - Name_E MDT form Hermal



Ministry of Health of Jordan
Ministry of Health of Jordan



الوزارة الصحة
وزارة الصحة

A-Patient ID:

1- Name : MDT

Sex:

MDT form

3- Medical number:

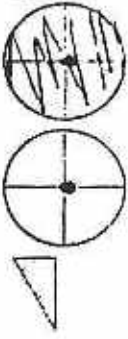
Female

83111230

Old Med :

2-ID number 26502041300101

Patient Age : 58 Y



Family History

Phone

01060028001

BMI

REASON OF MDT

NEW CASE

marital status

NBS Score

GROUP

Date of MDT

01/01/2023

Co morbidity

marital status

Menopausal status

Breast Imaging :

Clinical data

Mets Workup :

CT CAP: Multiple HFLs and multiple lung mets

BX:

LT IDC G11 11/22//*/

AT BAHEYA

IHC:

ER:

7/8

PR:

5/3

Her 2 Neu :

3

K167 :

T
cT4d

N

3

M

M1 liver, lung

Sonomammography

Recommendations:

Revise IHC

Echo. labs

Porta cath

Bone scan

MD1

OUTCOME & PLAN

For 1st line chemo p-~~if~~taxel + ...uzumab

V if HER2 positive on revision

MEDICAL ONCOLOGY

SURGERY

Psychiatric

SIGNATURE

RADIOLOGY

PATHOLOGY

Nutrition

about/blank

PATIENT NAME: Zeza MOHAMMED BAYYOMI ATEA	PATIENT ID #: 2022210311 ORDER #: 2011025,2011026 DATE OF SERVICE: 01-Dec-2022 03:06 PM
GENDER: Female AGE: 57 Y	

Report: - all studies
**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN
AND PELVIS**

1-CHEST

- Left breast marked skin thickening and edema with heterogenous enhancing parenchyma.
- Multiple pathological left axillary lymph nodes the largest measuring 28 x 20 mm.
- Multiple enlarged left supraclavicular and lower cervical lymph nodes the largest measuring 13 x 12 mm
- Multiple bilateral pulmonary nodules largest at lingula measuring 13 x 10 mm.
- No CT evidence of pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- No pericardial effusion detected.

2- ABDOMEN AND PELVIS

- The liver is enlarged showing multiple bilobar hypo enhancing focal lesions largest at segment IV b measuring 36 x 23 mm. No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- Bulky left supra renal.
- Normal appearance of the pancreas, spleen, kidneys, right adrenal, aorta, I.V.C and the pelvic organs.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.

11/11/2019

إهداء للطلاب المقدم من
 بشأن تقرير طبي للمريض: (نوع)
 محمد بيومي حسن
 رقم طبي
 بقسم

نتشرف بأن نوضح فيما يلي تقريراً عن حالته :

تاريخ ومساءة الدخول..... تاريخ ومساءة الخروج.....

best call for

Leads/Hits/Case IDs

الوقت: التاريخ: / /

١١١

اسم المريض ثلاثي:
رقم الملف:

تقرير طبي

السيد/.....

تحية طيبة وبعد،،

إيماء الطالب المقدم من
بقسم رقم طبي

بشأن تقرير طبي للمريض: (زويستاف يوسف زويستاف)
نشرف بأن نوضح فيما يلي تقريراً عن حالته :
تاريخ وساعة الدخول تاريخ وساعة الخروج

وبعد الكشف والفحوصات والمتابعة وجد انه:

Breast Cancer

تأخر في العلاج

✓

تحريراً في

الوقت: التاريخ: / /

الطبيب المعالج

.....

يعتقد

٥٥٦١/٩/٢٠٢١

اسم المريض ثلاثي:
رقم الملف:

تقرير طبي

السيد/.....

تحية طبية وبعد،،

إيماء للطلب المقدم من بشأن تقرير طبي للمريض: *زيتون محمد بن محمد*
بقسم رقم طبي

نُشرَف بأن نوضح فيما يلي تقريراً عن حالته :

تاريخ وساعة الدخول تاريخ وساعة الخروج

وبعد الكشف والفحوصات والمتابعة وجد انه:

Breast Cancer

fat

gem zar, carbo

تحريراً في

الوقت: التاريخ: / /

الطبيب المعالج

.....

يعتد ،،

c-c7/9/21